



Stomach: Gastritis, Ulcer & Cancer

Pathology GIT Block · MEDucation by Mattin Majid · Practice MCQs

HOW TO USE THIS SET

20 questions, mixed difficulty, 10 case-based. Attempt every question before checking the answer key on the last page.

● 4 Easy

● 10 Medium

● 6 Hard

1.

EASY

Tissue loss extending below the muscularis mucosa is termed:

- A. Ulceration
- B. Erosion
- C. Hyperplasia
- D. Atrophy
- E. Intestinal metaplasia

2.

EASY

A Curling ulcer is associated with:

- A. NSAID use
- B. Zollinger-Ellison syndrome
- C. Severe burns
- D. H. pylori infection
- E. Intracranial disease

3.

EASY

Approximately what proportion of gastric cancers are adenocarcinoma?

- A. 50%
- B. 60%
- C. 70%
- D. 80%
- E. >90%

4.

EASY

The commonest form of chronic gastritis is:

- A. H. pylori-induced gastritis
- B. Autoimmune gastritis
- C. Eosinophilic gastritis
- D. Chemical (reactive) gastritis
- E. Acute erosive gastritis

5.

MEDIUM

CASE

A 60-year-old woman has megaloblastic anemia and achlorhydria. Gastric biopsy shows atrophy with no organisms.

Which mechanism explains her anemia?

- A. Duodenal ulcer bleeding
- B. Antibodies against parietal cells with loss of intrinsic factor
- C. Chronic NSAID use
- D. H. pylori-induced hypochlorhydria
- E. Ileal resection

6.

MEDIUM CASE

A man with H. pylori gastritis has hypochlorhydria but no pernicious anemia.

Why does H. pylori gastritis not cause pernicious anemia?

- A. Parietal cell damage is not as severe as in autoimmune gastritis
- B. H. pylori affects only the duodenum
- C. B12 is absorbed in the stomach
- D. H. pylori increases intrinsic factor secretion
- E. Antibodies protect the parietal cells

7.

MEDIUM CASE

A patient with a severe head injury develops multiple acute gastric ulcers.

Which eponym applies?

- A. Marjolin ulcer
- B. NSAID ulcer
- C. Cushing ulcer
- D. Meckel ulcer
- E. Curling ulcer

8.

MEDIUM

Duodenal ulcers are how many times more common than gastric ulcers?

- A. Equally common
- B. Twice as
- C. Three times as
- D. Four times as
- E. Ten times as

9.

MEDIUM CASE

Endoscopy shows a round, sharply punched-out gastric defect with regular margins, radiating mucosal folds and a smooth clean base.

Which is the most likely diagnosis?

- A. Acute erosive gastritis
- B. Linitis plastica
- C. Peptic ulcer
- D. Ulcerated gastric carcinoma
- E. Hyperplastic polyp

10.

MEDIUM

In the histology of a chronic peptic ulcer, the four layers from the lumen down are:

- A. Inflammatory, necrotic, scar, granulation tissue
- B. Granulation tissue, necrotic, inflammatory, scar
- C. Necrotic, granulation tissue, inflammatory, scar
- D. Necrotic, inflammatory, granulation tissue, scar
- E. Scar, granulation tissue, inflammatory, necrotic

11.

MEDIUM

The commonest complication of peptic ulcer disease is:

- A. Pyloric obstruction
- B. Malignant transformation
- C. Hemorrhage
- D. Perforation
- E. Penetration

12.

MEDIUM CASE

A patient has an ulcer near the interface of the gastric body and antrum on the lesser curvature.

Which statement about this lesion is correct?

- A. It never bleeds
- B. It is the commonest type of peptic ulcer
- C. H. pylori is present in almost all such cases
- D. Malignant transformation, though very rare, is more likely than for a duodenal ulcer
- E. It usually involves the anterior wall

13.

MEDIUM CASE

Biopsy of the gastric antrum shows spiral bacilli on Warthin-Starry stain, neutrophils in the epithelium and lymphoid aggregates with germinal centers.

According to the slide diagram, antral-predominant infection leads to high acid production and:

- A. Linitis plastica
- B. Increased risk of duodenal ulcer
- C. Achlorhydria
- D. Increased risk of gastric adenocarcinoma directly
- E. Pernicious anemia

14.

MEDIUM

The commonest gastric polyp, arising in chronic gastritis, is:

- A. Juvenile polyp
- B. Peutz-Jeghers polyp
- C. Adenomatous polyp
- D. Fundic gland polyp
- E. Hyperplastic / inflammatory polyp

15.

HARD CASE

A 65-year-old man has early satiety and iron-deficiency anemia. Resection shows a rigid, leather-bottle stomach; histology shows signet ring cells.

Which statement about this tumour is correct?

- A. It has no known precursor lesion or risk factors
- B. It forms glands and usually arises from a precursor lesion
- C. It typically presents as an exophytic mass
- D. It is preventable by removing adenomatous polyps
- E. It is the intestinal type

16.

HARD

Which sequence describes the H. pylori route to gastric cancer?

- A. Atrophy, chronic gastritis, dysplasia, metaplasia, carcinoma
- B. Chronic gastritis, atrophy, intestinal metaplasia, dysplasia, carcinoma
- C. Intestinal metaplasia, chronic gastritis, atrophy, carcinoma, dysplasia
- D. Chronic gastritis, dysplasia, atrophy, carcinoma, metaplasia
- E. Dysplasia, chronic gastritis, atrophy, metaplasia, carcinoma

17.

HARD CASE

A patient with gastric adenocarcinoma has a hard left supraclavicular node and a periumbilical nodule.

Which eponyms describe these two findings?

- A. Sister Mary Joseph nodule and Krukenberg tumour
- B. Blumer shelf and Sister Mary Joseph nodule
- C. Krukenberg tumour and Irish node
- D. Irish node and Blumer shelf
- E. Virchow node (Troisier sign) and Sister Mary Joseph nodule

18.

HARD CASE

A patient on long-term PPI therapy has multiple gastric polyps; the family history includes FAP.

Which polyp type is associated with both PPI use and FAP?

- A. Fundic gland polyp
- B. Adenomatous polyp
- C. Inflammatory polyp
- D. Juvenile polyp
- E. Hyperplastic polyp

19.

HARD

Which dietary factor is associated with a decreased risk of gastric adenocarcinoma?

- A. Salted preserved food
- B. Large amounts of starch
- C. Smoked or cured fish and meat
- D. Fresh vegetables rich in vitamin C and other antioxidants
- E. Pickled vegetables

20.

HARD CASE

Endoscopy shows an ill-defined excavated gastric ulcer with irregular, heaped-up borders and a dirty base; histology shows malignant glands invading the muscular wall.

Which classification and pathway does this tumour fit?

- A. Diffuse type, from H. pylori-related metaplasia
- B. Intestinal type, usually from a precursor lesion with recognised risk factors
- C. Diffuse type, with no precursor
- D. Neither: it is an extranodal lymphoma
- E. Intestinal type, with no risk factors

ANSWER KEY & RATIONALE

Don't peek until you've committed to an answer on every question.

Q	Ans	Why
1	A	Ulceration extends below the muscularis mucosa; erosion is loss of superficial epithelium above it.
2	C	Curling = severe burns; Cushing = intracranial disease; stress ulcers = shock, sepsis, severe trauma.
3	E	Adenocarcinoma accounts for more than 90% of gastric cancers.
4	A	H. pylori-induced gastritis is the commonest form; autoimmune is <10% and chemical (reactive) is the third type.
5	B	Autoimmune gastritis: antibodies against parietal cells → loss of HCl and intrinsic factor → ileal B12 malabsorption → pernicious anemia.
6	A	H. pylori can cause hypochlorhydria but not achlorhydria or pernicious anemia because parietal cell damage is less severe.
7	C	Cushing ulcers occur in persons with intracranial disease.
8	D	DU are four times more common than GU.
9	C	Classic PUD: round to oval, sharply punched-out, regular margins with radiating folds and a clean base from peptic digestion of exudate.
10	D	N (necrotic), I (inflammatory), G (granulation tissue), S (scar/fibrosis).
11	C	Hemorrhage (occult or massive) is the commonest complication; malignant transformation is very rare.
12	D	GU sit at the body-antrum interface on the lesser curvature; DU are 4x commoner and have H. pylori in almost all cases. Malignant change is rare but more likely in GU.
13	B	Antral gastritis → high acid production → increased risk of DU. The other route (pangastritis → multifocal atrophic gastritis → intestinal metaplasia) raises adenocarcinoma risk.
14	E	Hyperplastic/inflammatory polyps are the commonest and arise in chronic gastritis.
15	A	Diffuse type (signet ring, infiltrative, desmoplastic linitis plastica): no precursor lesion and no known risk factors.
16	B	H. pylori → chronic gastritis → gastric atrophy → intestinal metaplasia → epithelial dysplasia → carcinoma.
17	E	Supraclavicular node = Virchow node (Troisier sign); periumbilical = Sister Mary Joseph nodule. Irish = left axillary; Krukenberg = ovary; Blumer = pouch of Douglas.
18	A	Fundic gland polyps are associated with FAP and chronic PPI use; adenomatous polyps are defined by dysplasia.
19	D	Risk falls with whole milk and fresh vegetables rich in vitamin C/antioxidants; it rises with starch, smoked or cured fish and meat, and pickled vegetables.
20	B	Gland-forming, ulcerated/exophytic = intestinal type, which usually arises from a precursor lesion and has risk factors.

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Recalled and source questions are treated as evidence of what gets asked, not as a source of correct answers — verify against your lecture notes.